



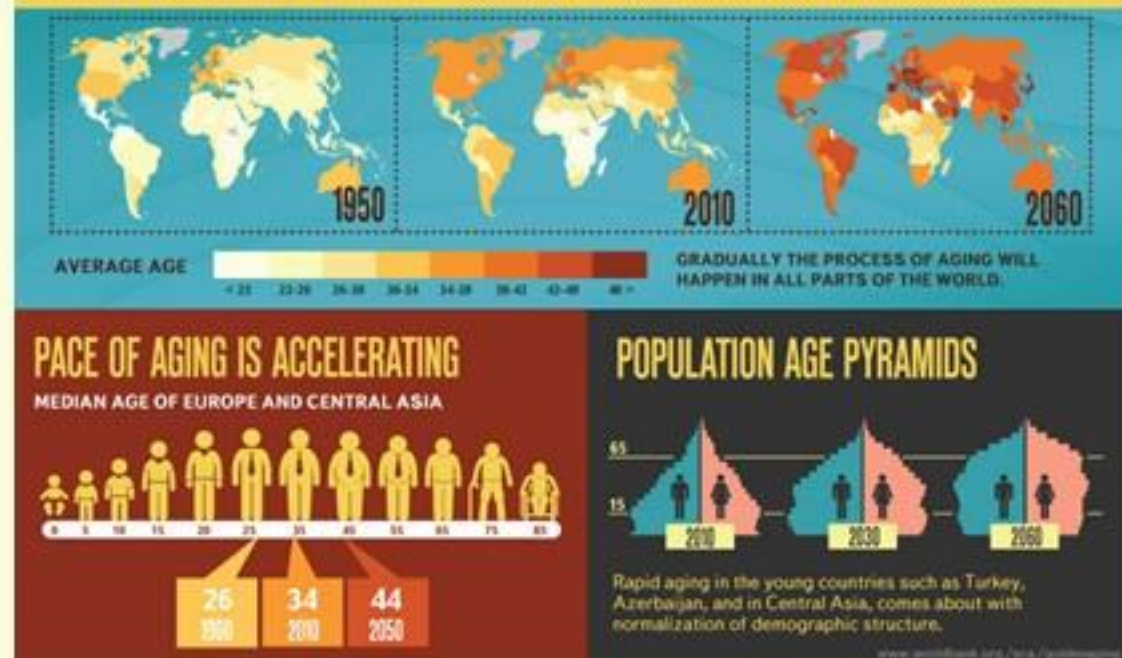
INDIAN SOCIETY

MODULE - 15

MISCELLANEOUS

- 1 Ageing is the change occurring in an individual as a result of passage of time.
- 2 Indian census considers those above 60 years as aged.
- 3 **WHO** further divides into:
 - Young old: 60-75 yrs
 - Old old: 75-85 yrs
 - Very old: 85yrs+

AGING SOCIETIES IN EUROPE AND CENTRAL ASIA



- 1 Elders who are above 60 years constitute 8% of Indian population and estimated to become 20% by 2050
- 2 India has world's second largest elderly population (UN population prospects)
- 3 DPSP 41 recognizes State's duty towards the aged
- 4 As per NSSO data, nearly 85% elderly in India dependent on others for day to day maintenance.

AGEING IN INDIA

India has **2,00,000** centenarians (100+ population).

Over **9 crore** elderly population in India in 2011 —only 12 other countries have a total population higher than that.

Percentage of 60+ population expected to increase from:

7.6%
(77 million) in 2000
to
20.6%
(324 million) in 2050.



In the same period, percentage of the 80+ population will increase from:

0.61%
(6.1 million) in 2000
to
3.06%
(48.2 million) in 2050.



48.2% of elderly are women, **58%** of them being widowed, divorced. **75%** of India's elderly live in rural areas and one-third live below the poverty line.

- **5.5 crore** go to sleep on an empty stomach every night—just about the population of the UK.
- An estimated **50 lakh** live alone – more than all of Australia.
- In 2040, within **30 years**, the grey population in India will double again.

Source: HelpAge India

AGEING

PROBLEMS FACED BY ELDERLY

- 1 Disintegration of traditional joint family and weakening of integrational ties, in which care of elderly was implicit.
- 2 Sociologically, ageing marks a form of transition from one set of social roles to another. In traditional agriculture based society, children followed parent's occupation and expertise and knowledge of the elderly was valued - offering a productive role for elderly.
- 3 However, changing mode of employment has led to elderly feeling loss of status, worthless and lonely.
- 4 Dual careers have 2 contrasting implications - working couples find support of parents in child care useful but high cost of living and healthcare in cities might discourage.



AGEING

PROBLEMS FACED BY ELDERLY

5 Lack of financial independence - due to retirement from formal sector or due to declining ability to work in unorganised sector.

6 Globalisation, modernisation, urbanisation resulted generation gap with new generations

7 Materialistic attitude of younger generation

8 Mental and physical health disabilities

9 Widowhood, segregation and prejudice aggravate frustration of elderly.



AGEING

PROBLEMS FACED BY ELDERLY

- 10** Women constitute higher % of elderly due to greater life expectancy at birth. Their problems amplify due to lack of financial independence, lack of ownership of assets, widowhood, 'empty nest syndrome' - which refers to the depression in the course of their diminishing role in the family.
- 11** Tradition of women marrying men older than them, increasing life expectancy of women, social disapproval of widow remarriage, patrilineal inheritance, etc. render women more vulnerable
- 12** Approximately 33% of India's elderly is BPL and 58% of them are widows.
- 13** Lack of geriatric, psychiatric care
- 14** Low access to retirement benefits and pensions



1 Maintenance and Welfare of Senior Citizens Act, 2007: Provides legal sanctions to rights of elderly, making it responsibility of children to take care of parents.

2 National Social Assistance Programme (NSAP)

3 CRPC and Hindu Adaptation and Maintenance Act 1956 recognize the right of parents to be supported by children.

4 National Centre for Ageing to train specialists for geriatric care and develop manuals for home care.

5 Integrated Program for Older Persons (IPOP): to establish day care centres, old age homes, mobile Medicare units etc.



- 1 According to Help Age India estimates, there are 728 such institutions in India.
- 2 Debate that increasing institutionalization of elderly would lead to erosion of desirable traditional family values.
- 3 However, there is also a need to accommodate increasing number of elderly with no support.



- 1 Promote awareness about concepts such as healthy ageing
- 2 Mohini Giri Committee asked to increase pensions and setting up of a department of senior citizens
- 3 Efforts should be made to strengthen family care. Reciprocal care and support within multigenerational families should be reinforced.
- 4 Strengthen legislative safeguards and institutional care.

- 5 Utilize the experience of elderly for societal welfare such as through counselling, mentoring, educating children etc.
- 6 Encourage organizations of elderly, which ensure their active involvement in policy and programme development (UN recommendation).



CHILD CARE AND INFANT MORTALITY

INFANT MORTALITY RATE

- 1 Infant Mortality Rate means the number of infants (below one year) deaths per 1000 live births.
- 2 Infant Mortality Rate which was 50 in 2009 and 47 in 2010 has declined to 44 as per the Sample Registration System (SRS) Bulletin, 2012.
- 3 In India, the infant mortality rate was as high as 140 per thousand live births in 1969
- 4 However, there are again variations all over the country.
- 5 For rural areas, the figure is 48 while for the urban areas it is 29.
- 6 In the bigger states, Kerala has the best rate of 12 while Madhya Pradesh is the worst with IMR of 59.
- 7 IMR is one of the indicators for the Millennium Development Goals (MDG) and a target of 28 has been set by the year 2015.

CHILD AND INFANT MORTALITY

CAUSES

- 1 Unavailability of Life-saving Equipment's:** low availability of life-saving equipment like ventilators, life support systems, radiant warmers, blood-pressure monitoring systems, etc. in major hospitals and primary/community/district health centres,
- 2 Pre-admission Causes:** These include, underweight babies who are more prone to infections within the first 48 hours of their birth. Ineffective management of the infections at hospitals increases the risk of infant mortality.
- 3 Insufficient Resources:** Because of the lack of funds/availability of insufficient funds, the required infrastructure is not in place and the systemic failures have become one of the prime reasons for infant deaths.
- 4 Abysmal Doctor to Patient Ratio**

EFFECTS OF POOR NUTRITION

India, Nigeria and Pakistan are home to almost half (47%) of all stunted children

**STUNTED CHILDREN IN THE WORLD**

INDIA
46.6 million



NIGERIA
13.9 million



PAKISTAN
10.7 million

largest number of stunted children in the world

India also accounts for the largest number of wasted children in the world: 25.5 million

Globally, 150.8 million children under five years are stunted and 50.5 million are wasted
38.3 million children globally are overweight

China, Indonesia, India, Egypt, US, Brazil and Pakistan have more than a million children overweight

CHILD AND INFANT MORTALITY

CAUSES

- 5 Unidentified Causes:** Sometimes the cause of child death are unknown. For example, in the case of Muzaffarpur (Bihar) deaths due to Acute Encephalitis Syndrome (AES)
- 6 Malnutrition:** More than half of Indian women are anaemic. According to National Family Health Survey-4: the prevalence of anaemia in women aged 15 to 49 years is 53%.
- 7 Type of Delivery:** Whether the child is born at home or in a facility also determines the infant survival rate. Infection risks are high in case of a non-institutional birth.
- 8 Lack of Immunity and Low vaccine Compliance level.**
- 9 Lack of education of the mother**
- 10 Age of the mother:** At the time of birth, age of mother plays an important role. For example, there exists an inverse relationship between the age of mothers and the incidence of anaemia in children



CHILD AND INFANT MORTALITY

GOVERNMENT INITIATIVES

1 India New-born Action Plan (INAP): It was launched in 2014 to make concerted efforts towards attainment of the goal of “Single Digit Neonatal Mortality Rate” and “Single Digit Still-birth Rate”.

2 National Health Mission (NHM): Its main programmatic components include health system strengthening in rural and urban areas for- Reproductive-Maternal-Neonatal-Child and Adolescent Health (RMNCH+A), and Communicable and Non-Communicable Diseases

3 Schemes like Janani Suraksha Yojana (JSY) and Janani Shishu Suraksha Karyakaram (JSSK), Pradhan Mantri Matru Vandana Yojana (PMMVY) etc. were started to promote institutional deliveries so as to reduce the prevalence of IMR.



CHILD AND INFANT MORTALITY

WAY FORWARD

INFRASTRUCTURE & SYSTEMIC CHALLENGES

- 1 The need of the hour is to become more preventive and build infrastructure beforehand rather than being curative as a healthy child develops a better nation.
- 2 Also, systemic gaps like referring and transporting critically ill-child, safely to the nearest AIIMS, need to be timely plugged.

INTEGRATED APPROACH

- Concerned ministries can collaborate with each other to ensure better coordination, convergence and holistic integration of different schemes, as done in POSHAN Abhiyan.
 - Funds allocated under Ayushman Bharat can be utilized in resolving the problem of infant death.

**POSHAN
Abhiyaan**PM's Overarching
Scheme for Holistic
Nourishment

सही पोषण - देश रोशन

CHILD AND INFANT MORTALITY

WAY FORWARD

POLITICAL WILL

- The availability of funds (from Centre) as well as its judicious use by the States is vital in effective implementation of the framed policies.

RESEARCH & ROLE OF APEX BODY

- To address the problem of unidentified causes, scientists, expert bodies, apex bodies like Indian Council of Medical Research (ICMR) need to work in tandem to identify the actual causes behind any outbreak in order to contain it.

PRIVATE PLAYER PARTICIPATION

- The involvement of private players is not an urgent requirement but their sincere engagement and complementing role to the state can ease down the burden of the government. The role of the state in delivering health to its people cannot be over emphasised.



INDIAN SOCIETY

TRANSGENDERS

- 'Transgender' refers to all those who differ in behaviour and appearance from the usual gender stereotypes.
- It includes transsexuals, transvestites (cross-dressers), intersexed individuals and gender queers.
- In the Indian context, it also includes social identities such as hijras, kinnars, aravanis, jogtas, Shivshaktis and aradhis.

1 Census 2011 records the population of 'others (people who do not identify themselves either as male or female)' at 4.87 lakh

2 While a 2011 survey by NGO Salvation of Oppressed Eunuchs put their number at 19 lakh.

3 Literacy rate is 46%



1 **Social stigma** - since birth they are segregated from society and discriminated against

2 **Education** - lack of access to formal schooling or special schools

3 **Underemployment** - represent less than 1% of workforce

4 **Legislations** such as Eunuchs Act of AP, have provisions discriminating transgenders.

5 Transgender identity is **not** recognised in Criminal law so there is no clarity on gender specific laws.

6 **Concerns on** right to property, adoption, marriage, pension etc.



1 India's first transgender sports meet was organised by Kerala. Kerala is the first state to formulate Transgender policy

2 Learn from international examples - Ireland and Denmark allow for self-determination of gender without medical treatment

3 NALSA v. Union of India, 2014:

- SC affirmed the fundamental rights of transgender persons.
- The court gave a series of directives to the government to institute welfare measures for transgender persons, including affirmative action.
- It also directed that the Expert Committee Report prepared by the Ministry of Social Justice and Empowerment (MSJE) be implemented.



THE TRANSGENDER PERSONS BILL 2019

- 1** Definition of a transgender person:
 - The Bill defines a transgender person as one who is
 - (i) Neither wholly female or male;
 - (ii) A combination of female and male; or
 - (iii) Neither female nor male
- 2** Prohibition against discrimination
- 3** Certificate of identity for a transgender person

- 4** National Council for Transgender persons: to advice government and monitor implementation.

TRANSGENDER PERSONS BILL 2019



THE TRANSGENDER PERSONS BILL 2019

5 Criticism:

- Compulsory certification coupled with onerous procedure for certification stands in violation of the self-identification principle.
- Unlike the previous private member bill, the new bill does not provide the definition of discrimination.
- The new bill does not cover reservation provision which was earlier dealt in the previous bill and even the NALSA judgement.

Empowering Transgender Persons

Transgender Persons (Protection of Rights) Bill 2019 Approved



To mitigate the **stigma, discrimination & abuse** against transgender persons



To bring transgender into the **mainstream** of the society & make them productive members



To bring **greater accountability of all stakeholders** dealing with Transgender persons, and make them more responsive

DISABLED

- As per Census 2011, PwDs are 2.21% of the total population (could be grossly underestimated due to prevalent stigma)
- 70% of the disabled population was rural. 45% of all PwDs in India are illiterate.

ISSUES

- 1 Disability is not being measured properly in India. Not all census measures it and has different definitions.
- 2 India looks at disability from medical or pathological angle only. Most developed countries look from social angle.
- 3 Unfriendly public infrastructure
- 4 Social stigma leads to huge under reporting.
- 5 Census depends on self-reporting of disability; this may leave out mental disability.
- 6 GST will be imposing a tax rate of 18% on aids and appliances of disabled which were 0 rated for the past 10 years.



DISABLED

STEPS TAKEN

- 1 India is a signatory to UN Convention on the Rights of Persons with Disabilities and the Incheon Strategy (to “Make the Right Real” for PwDs in Asia and the Pacific.)
- 2 National Action Plan for Skill Development covers PwD
- 3 Name changed to 'Divyang' instead of Viklang.
- 4 Kerala became first state to conduct a census of its own called Kerala Disability Census for 2014- 15.
- 5 Accessible India Campaign: covers built up environment, transportation eco-system and information & communication eco-system
- 6 Developing Accessibility Index and Special University for PwD.



RIGHTS OF PERSONS WITH DISABILITIES ACT 2016

- Reservation increased from 3 to 4%
- Disabilities covered under the act increased from 7 to 19
- Rights and entitlements to disabled persons and friendly access to all public places
- National and State Commissions for persons with disabilities along with Central and State Advisory Boards on disability, national and state level funds
- Plenary guardian for mentally disabled
- Disability benchmark is fixed at 40%
- Right to free education up to 18 years.

Parliament passed Rights of Persons with Disabilities Bill - 2016

Salient Features



Reservations in vacancies increased to 4% from 3%

Reservation in higher education and Government jobs

Right to free education for children of 6 to 8 years with disabilities

Educational institutions funded/ recognised by government are required to provide inclusive education

Reservation in allocation of land, poverty alleviation scheme etc.

3 blood disorders added - Thalassemia, Hemophilia and Sickle Cell disease

DISABLED

STEPS TAKEN

NITI ACTION PLAN

- 1 States should be encouraged to develop their own disability policies similar to the Comprehensive Disability Policy Framework of Chhattisgarh.
- 2 Awareness campaign to remove stigma and difficulties in data collection
- 3 Accessibility, Education:
 - NCERT study found that disabled children in schools across states still face serious infrastructure and pedagogy handicaps.
 - There is a need to Include mandatory module on sensitization in teacher training.

- 4 Unique Disability Identity Card (UDID) Project should be rolled-out, eventually create an electronic database of PwDs across the entire country.



ENVIRONMENTAL MOVEMENTS

1 Environmental movement is a type of social movement that involves individuals, groups and coalitions that perceive a common interest in environmental protection and act to bring about changes in environmental policies and practices.

2 The genesis of environmental movement in India can be traced back to the early twentieth century when people protested against the commercialization of forest resources during the British colonial period.



1. BISHNOI MOVEMENT

- Movement was led by Amrita Devi in which around 363 people sacrificed their lives for the protection of their forests.
- This was the first of its kind to have developed the strategy of hugging or embracing the trees for their protection spontaneously.

2. THE CHIPKO MOVEMENT

- Non-violent agitation in 1973 that was aimed at the protection and conservation of trees, but perhaps, it is best remembered for the collective mobilisation of women for the cause of preserving forests, which also brought about a change in attitude regarding their own status in society.
- The name of the movement 'chipko' comes from the word 'embrace', as the villagers hugged the trees and encircled them to prevent being hacked.



MAJOR ENVIRONMENTAL MOVEMENTS IN INDIA

3. NARMADA BACHAO ANDOLAN

- Against the Narmada River Valley Project.
- It has drawn upon a multiplicity of discourses for protests such as: displacement risks and resettlement provisions; environmental impact and sustainability issues; financial implications of the project; forceful evictions and violations of civil liberties; issues pertaining to river valley planning and management etc.



4. APPIKO MOVEMENT

- One of the forest-based environmental movements in India.
- The movement took place in Uttara Kannada district of Karnataka in the Western Ghats.
- The movement created awareness among the villagers throughout the Western Ghats about the ecological danger posed by the commercial and industrial interests to their forest which was the main source of sustenance.



MAJOR ENVIRONMENTAL MOVEMENTS IN INDIA

5. SILENT VALLEY MOVEMENT

- Silent Valley Movement in Kerala was against the construction of a hydroelectric dam on the river Kunthipuzha under the Kudremukh project.



6. TEHRI DAM CONFLICT

- It is one of the most protracted environmental movements in recent years.
- The major issues of the movement include- seismic sensitivity of the region, submergence of forest areas along with Tehri town etc.
- However, along with these ecological concerns, there were also economic and identity issues associated with these environmental movements.



ENVIRONMENTAL MOVEMENTS

ECONOMIC ISSUES

- 1 Villagers relied on the forest to get firewood, fodder and other daily necessities.
- 2 The livelihood needs of poor villagers were put at stake against the government's desire to generate revenues from selling timber.
- 3 The need for infrastructural development attracted many foreign logging companies, who were eyeing the vast forest resources.
- 4 This led to denying of villager's control over these natural resources on which they relied for both food and fuel.
- 5 Widespread flooding of the villages and forest areas, which was attributed to the mismanagement due to commercial logging and construction of big dams, led to displacement of villagers thereby losing their means of livelihood.



ENVIRONMENTAL MOVEMENTS

IDENTITY ISSUES

- 1 The villagers valued the forest for their own sake and were of the view that their existence and identity is closely linked to the forest.
- 2 They were able to perceive the link between their victimization and the denuding of mountain slopes by commercial interests.
- 3 Women, being solely in charge of cultivation, livestock and children, suffered the most due to floods and landslides.
- 4 The message of the environmental movement leaders made a direct appeal to them.
- 5
 - Women were given no share in the decision-making process, public power and political activities like men.
 - These movements provided them with the opportunity to raise their concerns and fight for their rights.



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